This Patient Group Direction (PGD)is intended only for registered healthcare professionals who have been named and authorised by their organisation to practice under it.

Patient Group Direction
for the administration of Azithromycin for the treatment of Traveller's Diarrhoea

Patient Group Direction, version 007, issued 30 September 2026. Supersedes Version 006, 14 September 2026.

Owner and authorising organisation: Get Real Health Limited, company number 12744898, registered office Zenvite Health Unit 23, St. Asaph Business Park, St. Asaph, Denbighshire, LL17 0LJ. This Patient Group Direction is authorised on behalf of Get Real Health Limited by its Medical Director and its Head Pharmacist, whose signatures for this version are on the authorisation page at the end of the document. It applies to the registered healthcare professionals named under it by a pharmacy or clinic that has adopted it through a Get Real Health service agreement, at the premises stated in that adoption, and to no other organisation.

Summary of the governing guidance for travellers' diarrhoea: NaTHNaC / TravelHealthPro travellers' diarrhoea factsheet, the BNF, and the SmPCs of the products supplied. There is no NICE CKS topic on travellers' diarrhoea.

Overview

- **Traveller's diarrhoea** is defined as **≥3 loose or watery stools in 24 hours**, often accompanied by **abdominal cramps, nausea, or vomiting**, occurring during or shortly after travel, typically to low- and middle-income countries.
- Most cases are **mild**, **self-limiting**, and resolve within **3–5 days**.
- Common pathogens: **Enterotoxigenic *E. coli* (ETEC)**, **Campylobacter**, **Shigella**, **Salmonella**, and viruses.

Risk Factors

- Travel to high-risk areas (e.g. South Asia, Sub-Saharan Africa, Latin America)
- Poor sanitation and hygiene
- Eating street food or raw/undercooked food
- Young age, immunocompromise, PPI use

Prevention Advice

Food and Water Hygiene

- Drink **bottled or purified water**
- Avoid **ice, salads, unpeeled fruit, undercooked meat/seafood**
- Wash hands regularly, especially before eating

Prophylactic Antibiotics

- **Not routinely recommended** due to resistance risk
- May be considered for:
  - High-risk travellers (e.g. immunocompromised)
  - Short-term, high-risk exposure (e.g. disaster relief workers)

Self-Treatment Advice

Mild to Moderate Diarrhoea

- **Oral rehydration** (key priority)
- **Anti-motility agents** (e.g. **loperamide** 4 mg initially, then 2 mg after each loose stool, max 16 mg/day)
- **Loperamide** not for:
  - Children <12
  - Blood in stool
  - Fever

Short-course Antibiotics (if moderate/severe symptoms or essential travel plans disrupted):

- **Azithromycin 500 mg once daily for 1–3 days** (preferred in most areas)
- **Ciprofloxacin 500 mg BD for 1–3 days** (not recommended in SE Asia due to resistance)
- Only provide for **self-start** in **pre-travel consultation** if appropriate

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When to Seek Medical Help

- **Fever**, **bloody diarrhoea**, or **severe abdominal pain**
- **Persistent vomiting** or **inability to stay hydrated**
- Diarrhoea lasting **>14 days**
- In **infants, elderly, or immunocompromised** patients

Post-Travel Considerations

- If diarrhoea persists after return:
  - Investigate for **giardiasis**, **amoebiasis**, **chronic bacterial infection**, or **IBS post-infection**
  - Send stool samples if symptoms persist >7–10 days

Patient Group Direction

for the supply/administration of

Azithromycin

for the treatment of

Traveller's Diarrhoea

Healthcare professionals covered and training

	Healthcare professionals allowed to work under this PGD and requirements
Qualifications and Registration	● Pharmacist registered and practicing with the GPhC ● Pharmacy technician registered and practicing with the GPhC
Training and assessment	• Pharmacists and Pharmacy Technicians must have completed training relevant to this condition to use this PGD. This training will be documented and overseen by the Get Real Health team. ● All users must be familiar with the SPC for the products as well as BNF advice and any applicable national guidelines ● All users must have completed training in working under Patient Group Directions, for example the CPPE PGD e-learning and e-assessment or an equivalent, and have been assessed as competent to work under a PGD before first working under this one; previous experience of working under a PGD is not required ● All users should complete the Get Real Health training for this PGD before first working under it, or have completed equivalent training elsewhere, for example another provider's training for the same service or substantial recent experience of delivering it under a PGD, and must read this PGD in full before first working under it. The date the training was completed, and for equivalent training what it was, is to be recorded in the Get Real Health training record or in the employing organisation's own training record. The employing organisation remains responsible for being satisfied that each practitioner it authorises under this PGD is competent to do so. ● Must work in compliance with the SOPs of their own employer ● All users must have appropriate indemnity in place to cover this service

Further training and responsibilities	● All users must be up to date with CPD requirements and appraisal ● All users must be up to date with MHRA and safety alerts with regards to medical products ● All users must understand the concepts of capacity and consent and be aware of the Mental Capacity Act 2005.

PGD Indication	Azithromycin is indicated for the treatment of moderate to severe traveller's diarrhoea, particularly when caused by likely bacterial pathogens and where fluoroquinolone resistance is a concern. Supply is normally made at a pre-travel consultation as a standby course, for the traveller to start only if moderate to severe symptoms develop during travel.
Inclusion criteria	- Adults aged 18 years and over. - Recent or planned travel to high-risk regions for traveller's diarrhoea (e.g., South Asia, Africa). - No known contraindications to macrolide antibiotics. - Informed consent obtained and patient understands when and how to self-treat. - Suitable for self-start treatment during travel if symptoms develop.
Exclusion criteria	- Known allergy to azithromycin or other macrolide antibiotics. - Severe liver disease or significant hepatic dysfunction. - Concomitant medications known to prolong the QT interval. - Bloody diarrhoea, high fever (a temperature of 38 C or above), or signs of systemic illness. - Current symptoms that have lasted more than 72 hours without improvement: refer, do not supply. A standby course is not for a patient who is already unwell.
Cautions	- Take tablets with food to reduce GI upset. - Advise patient to maintain hydration and recognise signs of severe illness. - Counsel on seeking local medical attention if symptoms worsen or persist. - Stop treatment if hypersensitivity or serious side effects occur.
Actions if patient is excluded or declines treatment	Advise on alternative treatment options and how these can be accessed. Document any advice given and the decision reached. Inform or refer to the GP as appropriate

Details of the medicine

Name, form and strength of medicine	Azithromycin 500 mg tablets.
Legal category	POM
Storage	Store below 25°C in original packaging.
Route/method of administration	Oral administration, preferably with food.
Dose and frequency	500 mg once daily for 1 to 3 days, depending on clinical severity.
Quantity to be administered and/or	One to three 500 mg tablets depending on symptom severity

supplied	
Maximum or minimum treatment period	Maximum treatment course is 3 days.
Adverse effects	Nausea, abdominal pain, diarrhoea, headache. Rare: hepatotoxicity, allergic reactions, QT prolongation. Refer to SmPC for full details.

Yellow card reporting	Report suspected adverse effects via https://yellowcard.mhra.gov.uk and advise the traveller to consult a healthcare professional if symptoms worsen.
Records to be kept	Record the following information: ● that valid informed consent was given ● name of individual, address, date of birth and GP ● name of healthcare practitioner ● name and brand of medication ● date of supply, dose, form and route, quantity supplied, batch number and expiry date ● advice given, including advice given if excluded or declines treatment ● details of any adverse drug reactions and the actions taken, and that the supply was made under this PGD Records should be signed and dated. All records should be clear, legible and contemporaneous. Electronic records can be made. A record of all individuals receiving treatment under this PGD should also be kept for audit purposes as follows: ● Adults aged 18 years and over - keep records for 8 years ● Children aged under 18 years -keep records until the 25th birthday (if the patient was 17 years when treatment was finished, keep records until the 26th birthday)

Patient information

Written information to be given to patient or carer	Supply the patient information leaflet (PIL) provided with the medication.
Follow-up advice to be given to patient or carer	Start the course only if moderate to severe diarrhoea develops during travel (three or more loose stools in 24 hours with distressing symptoms). Seek local medical help urgently for blood in the stool, fever, severe abdominal pain, persistent vomiting or inability to keep fluids down, or if symptoms have not improved within 24 to 72 hours of starting azithromycin. Seek medical advice for any diarrhoea lasting more than 14 days, and on return if symptoms persist.

Key references

- NICE Medicines Practice Guideline 2 (MPG2): Patient Group Directions. Published March 2017 https://www.nice.org.uk/guidance/mg2
- NICE MPG2 Patient group directions: competency framework for health professionals using patient

group directions. Updated March 2017 https://www.nice.org.uk/guidance/mpg2/resources

- NaTHNaC TravelHealthPro, Travellers' diarrhoea factsheet; BNF; Summaries of Product Characteristics for the products supplied, current versions
- British National Formulary (BNF) Key information on the selection, prescribing, dispensing and administration of medicines BNF (British National Formulary) | NICE
- Summary of Product Characteristics for drug

Agreement to practise by the Registered Healthcare Professional

By signing this PGD you are confirming that you agree to its contents and that you will work within it. PGDs do not remove inherent professional obligations or accountability. It is the responsibility of each healthcare professional to practise only within the bounds of their own competence and professional code of conduct.

Name and address of pharmacy premises / clinic premises to which this PGD relates			
Name of healthcare professional	Job title	Registration number	Signature
Valid from date		Expiry date	
30/9/26		31/7/27	

PGD adoption and authorisation by the employer/clinical lead

The employer has ensured that the person using this PGD, has the knowledge and skills to safely provide the service which will encompass the supply or administration of a medication or vaccine.

This section must be signed by the superintendent or clinical lead responsible for this service. To be completed by the adopting pharmacy. These are the adopting organisation's own signatories and must not be Get Real Health staff.

Name of superintendent / clinical lead	Job title & name of organisation	Signature	Date

Change history to version 002

Later versions are recorded in the Version and change record at the end of this document.

Version number	Change details	Date
001	Development & issue of new PGD	25th June 2025
002	Reviewed and authorised for the GRH platform; signed by Medical Director	13th July 2026

Version and change record

Version: v007

Issued: 30 September 2026

Supersedes: Version 006, 14 September 2026

Valid from: 30 September 2026. Expiry: 31 July 2027. These dates govern this version and supersede any earlier date printed elsewhere in this document.

Changes in this version:

Training before first use, applied to every Get Real Health PGD on 30 September 2026: the staff section now states that each practitioner should complete the Get Real Health training for this PGD, or equivalent training elsewhere, before first working under it, must read the PGD in full before first use, and that the date of completion is recorded. Training is recorded; it is not a condition of access, and the employing organisation remains responsible for being satisfied of competence. No clinical content changes.

Previous version records

Version: v006

Issued: 14 September 2026

Supersedes: Version 005, 11 September 2026

Valid from: 14 September 2026. Expiry: 31 July 2027. These dates govern this version and supersede any earlier date printed elsewhere in this document.

Changes in this version:

Identity and training, applied to every Get Real Health PGD on 14 September 2026 after two adopting pharmacies wrote: the Get Real Health logo is in the page header; the organisation that owns and authorises this direction, Get Real Health Limited, is named under the strapline with its company number and registered office; the requirement that practitioners must previously have used a PGD is replaced by a requirement for PGD training and a competence assessment, so that a newly registered pharmacist or technician who has completed the training may work under it; and the earlier change history table is headed with the versions it covers.

Previous version records

Version: v005

Issued: 11 September 2026

Supersedes: Version 004, 11 September 2026

Valid from: 11 September 2026. Expiry: 31 July 2027. These dates govern this version and supersede any earlier date printed elsewhere in this document.

Changes in this version:

Decision 2: high fever in the exclusion criteria is now defined as a temperature of 38 C or above, the sepsis threshold used across the Get Real Health PGDs; the consultation tool applies the same figure.

Decisions of the authorising signatories, 11 September 2026: the questions raised by the reviews of 10 and 11 September were put to the Medical Director and answered; each answer is written into this document above and, where the answer set a rule, into the consultation tool. Text drafted from a source for the Head Pharmacist to confirm is marked as such in the change lines.

Previous version records

Version: v004

Issued: 11 September 2026

Supersedes: Version 003, 11 September 2026

Valid from: 11 September 2026. Expiry: 31 July 2027. These dates govern this version and supersede any earlier date printed elsewhere in this document.

Changes in this version:

The indication now states that the supply is normally a standby course made at a pre-travel consultation, matching the inclusion criteria and the guidance summary.

The 72 hour exclusion now reads as a supply exclusion: a patient who is already unwell for more than 72 hours without improvement is referred, not supplied.

The name, form and strength row now states azithromycin 500 mg tablets, as Schedule 16 requires.

The records row has its punctuation restored and now asks for the batch number and expiry date.

The follow-up advice row, which carried the skin infection template wording about 3 to 4 weeks, now gives the self-start and when-to-seek-help advice appropriate to a 3 day standby course.

Wording corrections from the tool alignment and adversarial review of the consultation tools, 11 September 2026. Each correction resolves a contradiction inside the document or a plain error, taking the safer reading; no indication is widened, no dose raised and no exclusion removed. Questions that need a clinical decision are recorded separately and are not changed here.

Previous version records

Version: v003

Issued: 11 September 2026

Supersedes: Version 002, 13 July 2026

Valid from: 11 September 2026. Expiry: 31 July 2027. These dates govern this version and supersede any earlier date printed elsewhere in this document.

Changes in this version:

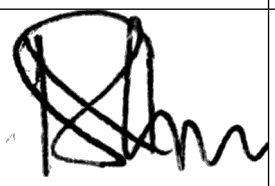
Reissued so that both signatories sign the same version on the same date. Version 002 carried the Medical Director's signature dated 13 July 2026 beside a Head Pharmacist signature dated 25 August 2025, from version 001.

The guidance summary and references are attributed to NaTHNaC, the BNF and the SmPCs; there is no NICE CKS topic on travellers' diarrhoea.

Structural clean-up applied to every live Get Real Health PGD on 11 September 2026, following the adversarial review of 10 September: the adopting pharmacy's authorisation block is blank, for the pharmacy's own signatories; one signed authorisation page for this version replaces the earlier signature blocks; every validity cell states this version's dates (valid from 11 September 2026, expiry 31 July 2027); narration about earlier versions, the consultation tool and individual customers is removed from the body of the document; the change records of earlier reissues are carried below as previous version records. No clinical criterion changes unless listed.

Signed on behalf of Get Real Health

Version v007, 30 September 2026.

Name	***Qualification***	***Signature***	***Date***
Nitin Shori	Medical Director GMC: 6047293		30 September 2026
Chris Pilkington	Head Pharmacist GPhC: 2046322		30 September 2026

Both authorising signatories reviewed this version together on 30 September 2026 and gave their authorisation for it to be issued. Signatures were applied digitally on their joint instruction, which is the established process for Get Real Health PGDs. This version is not valid without both signatures.